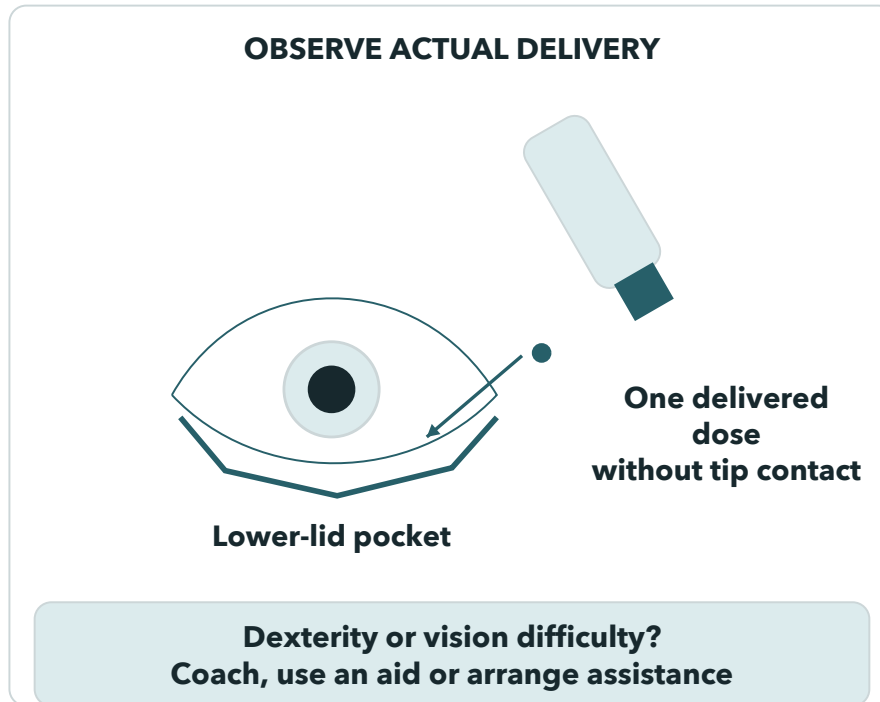


Glaucoma treatment failure: check why

Delivery, duration of response and surface tolerance are actionable causes.

The dose must reach the eye

A | Observe the patient instilling a drop



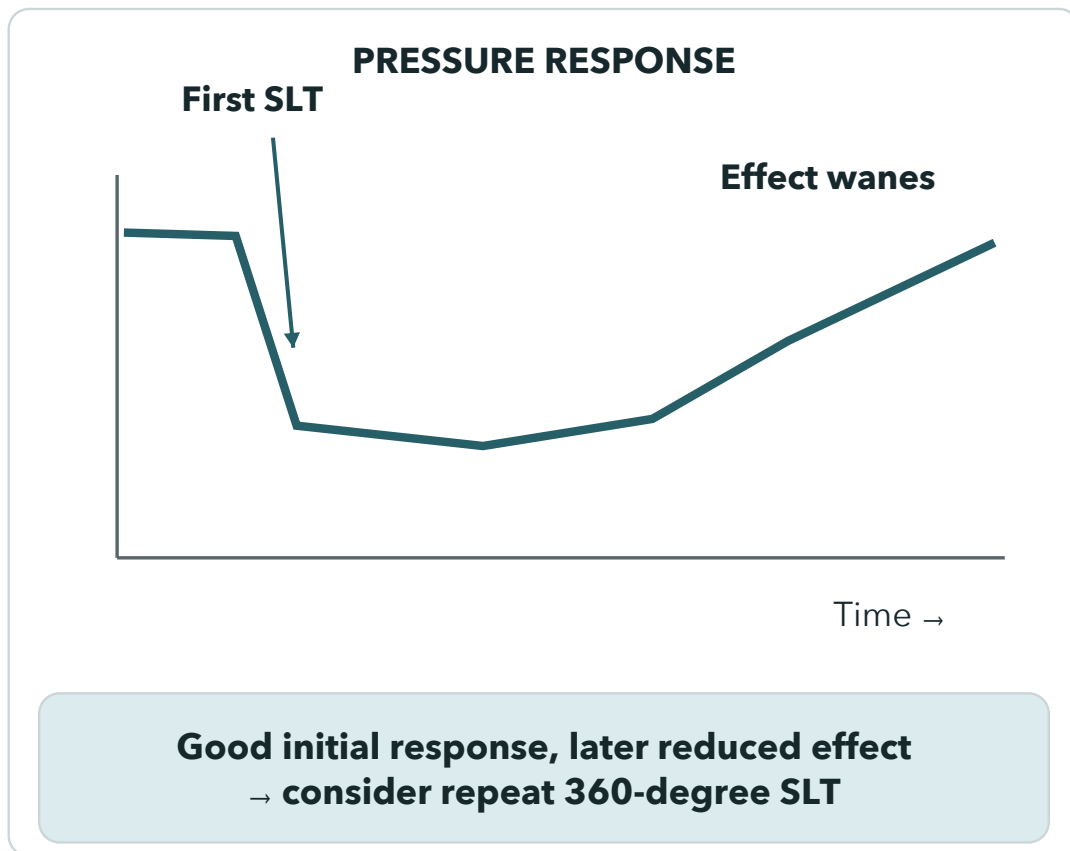
Original instillation schematic. Teach a workable technique and consider aids or assistance when dexterity or vision limits delivery.

Ask before adding another bottle

Problem	Possible correction
Missed or misdirected doses	Nonjudgmental history, observed technique, reminders, aid or assistance.
Painful preserved regimen	Review cumulative surface burden; consider preservative-free or suitable alternative therapy.
Adequate delivery; high pressure	Reassess target and treatment efficacy, then escalate as appropriate.
Threatened vision	Troubleshooting must not delay urgent pressure control when the nerve is at risk.

A good laser response can fade

B | Initial success, later loss of effect



Original response curve. NICE permits consideration of a second 360-degree SLT after a successful first response wanes.

Keep the review individual

Recheck the angle, current pressure, structural and field trends, and treatment preference. Repeat SLT can be suitable in nonadvanced disease, but response is not guaranteed and drops may still be needed. Significant progression or advanced damage can change the treatment pathway. Surface intolerance is a reason to reconsider formulation and delivery burden, not to abandon pressure control. Explain the next review and symptoms that need earlier assessment whenever the regimen changes. Ask the patient to show the usual technique with the actual bottle. A dose landing on the cheek is a delivery problem despite perfect reported timing. If correct delivery still leaves pressure above the agreed target, the next decision concerns efficacy and escalation.